

إسم المريض ربيعاً : نهله شعبان عبدالحكيم حسن
المحافظة : المنوفية الحي / المدينة : السادات
رقم البطاقة : 28502120102703 رقم التليفون : 01114993044
العمر(تاريخ الميلاد) : 12-02-1985 المهنة : ربه منزل
العنوان : الخطاطبه
تاريخ تسجيل الإستمارة : 03-10-2026

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه
التوقيع: (-----)

التعليق على التشخيص : -----

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 نفقات :
 ملاحظات :

رئيس
المجلس الطبي المتخصص

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Name: نهله شعبان عبدالحكيم حسن National ID: 28502120102703	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



 جمهورية مصر العربية
 وزارة الداخلية
 بطاقة تحقيق الشخصية

تيمه
 شعبان عبد الحكيم حسن
 اليفقه - الخفاطيه
 مركز المسالك - النورانية

٢٨٥٠٢١٢٠١٠٢٧٠٣
 ١٩٨٥/٠٣/١٢
 IU6060149



 ٢٠٢١/٠٣ ٢٨٥٠٢١٢٠١٠٢٧٠٣
 مليم حجرة
 أنش
 يوسف محمد زكي يوسف أبو العلا
 البطاقة سارية حتى ٢٠٢٨/٠٣/٠٧



 ١٢

Dr. MAKRAM MILAD
Professor of Pathology , Calro University
Pathologist and Cytologist

دكتور مكرم ميلاد
أستاذ التحاليل الباثولوجية للأنسجة والأورام
ورئيس قسم الباثولوجيا بكلية طب قصر العيني

Name : السيد نهله شعبان عبد الحكيم

Date : 16/8/2024

Age : 38

Referred by Prof. Dr. Abdelghany Mohammed

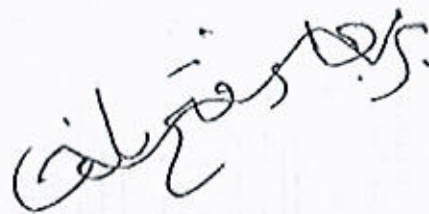
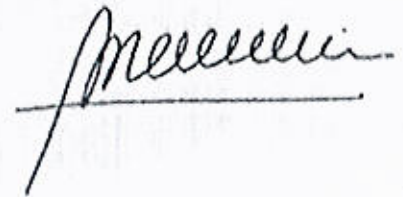
Clinical diagnosis: Left mammary carcinoma, M.R.M.

PATHOLOGY REPORT

M.R.M. of the left breast. No nipple retraction. Mild peau d'orange of the skin of the breast. Slicing of the breast revealed an irregular, hard and gritty mass at the L.O.Q. of the breast measuring 35x20x20 mm. in diameters. The mass is adherent to the surrounding breast tissue. The left axillary fat contains 14 palpable lymph nodes, the largest sentinel node measured 20 mm. in diameter. Histological examination of the breast mass revealed a picture of invasive ducto-lobular carcinoma (grade III) arising from the terminal ductal lobular units. The carcinoma tissue shows dense stromal desmoplasia and lymphocytic infiltrate. 6 of the axillary lymph nodes including the sentinel node show metastases with capsular invasion. All the surgical margins of the breast are free from any tumor tissue extension.

Diagnosis :

Invasive ducto-lobular carcinoma of the left breast (grade III).
Axillary lymph node metastases with capsular invasion (6 / 14).
M.R.M. with free surgical margins . Stage : T2 N2.



Address: 63 Nobar Str. , Bab El Louk , Daily from 2-7 PM , Tel: 27945620

Name: نهله شعبان عبد الحكيم		
MR. : 4344		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشر بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient presents with a history of left breast cancer diagnosed on 16/8/2024, staged T2 (3.6 × 2 × 2 cm) N2 (6/13) IDC, ER 5/8, PR 0/8, HER2-negative, Ki67 5/8. Imaging at the time of diagnosis showed multifocal malignant masses in the lower quadrants with ipsilateral infiltrated nodes. The multidisciplinary team recommended adjuvant therapy including four cycles of taxotere, followed by zoladex, then a PORT, and aromatase inhibitor therapy with zoladex for five years plus zoledronic acid for six months after dental assessment. The patient has been on FEMARA since 16/4/2025 and received a free CT chest/abdomen/pelvis on 17/12/2025. Calcium supplementation and a single dose of alpha zoladex 10.8 mg were administered. The patient returned on 01/03/2026 for a zoladex 10.8 mg injection and arimidex, with supportive therapy for three months. On 19/04/2026 the oncology team noted that the patient is on zoladex and arimidex from 20/40/2025, with the next zoladex dose due 24/5/2026, and is scheduled for zoledronic acid and arimidex. On 22/08/2026 the internal medicine note states that the patient is continuing arimidex with supportive care and zoladex for three months, with the next visit scheduled for 15/11/2026.		
تحريراً في ,		
	الوقت :	التاريخ : 21-09-2026
	الطبيب المعالج : ايه مدحت بدوي محمد	
يعتمد ,		

Patient Name : الأستاذة / نهلة شعبان عبد الحكيم
Patient No. : 28502120102703
Lab No. : SR-1268/24
Sex : Female
Age : 39 Y
Referring Doc :

Request No. : 13007626
Request Date : 04/09/2024
Verification Date : 22/09/2024
Financial Category : أورام دار السلام - نفقة الدولة
Department : Out Patient
Registered by : Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide Revision

Gross Examination:
5 paraffin blocks and 5 slides labeled by (495/24) were received.

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade (differentiation+ nuclear pleomorphism+ mitosis): Grade II (3+2+1)
* DCIS: Focal.
* Angiolymphatic invasion: Positive.
* Perineural invasion: Couldn't be detected.
* Lymph node: Positive in (6/13).
* Skin: Free.

Diagnosis:
Left breast cancer, Slide revision:
Invasive duct carcinoma, Grade II with focal DCIS.
Free skin.
Positive lymph nodes (6/13).
For further IHC (ER, PR, Her2, Ki67).

Signature
Dr. Rana Fathy.
Dr. Sagda Elkady.

Verified by :

Printing Date : 22/09/2024 10:57:23

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadema - Cairo, Egypt

CAP
ACCREDITED

Patient Name	: الأستاذة / نهلة شعبان عبد الحكيم	Request No.	: 13007626
Patient No.	: 28502120102703	Request Date	: 04/09/2024
Ref No.	: SR-1268/24	Verification Date	: 30/09/2024
Sex	: Female	Financial Category	: نفقة الدولة - اورام دار السلام
Age	: 39 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Left breast cancer.

Nature of Specimen:
Slide and block revision.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:
- PT (Positive control showed appropriate reactivity):
- ER: Moderate nuclear reaction in 15% of tumor cells (score 5/8).
- PR: No nuclear reaction in tumor cells (score 0/8).
- Her2: Weak incomplete membrane reaction in >10% of tumor cells (score 1+).
- Ki67: 15% nuclear reaction of tumor cells.

Conclusion:
Left breast cancer, slide and block revision:
Invasive duct carcinoma, Grade II with focal DCIS.
Free skin.
Positive lymph nodes (6/13).
PT:
- ER: Positive (score 5/8).
- PR: Negative (score 0/8).

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Patient Name الأستاذة / نوله شعبان عبد الحكيم
Patient No. 28502120102703
Path No. SR-1268/24
Sex Female
Age 39 Y
Referring Doc.

Request No. 13007626
Request Date 04/09/2024
Verification Date 30/09/2024
Financial Category أورام دار السلام - نفقة الدولة
Department Out Patient
Registered by Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Her2: (Score 1+).

Ki67: Positive in 15% of tumor cells.

Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, 1-10%; 2, between 11% and 33%; 3, between 34% and 66% and 4, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Prof. Gamal Abdelatty
Dr. Marwa Mahmoud
Dr. Menna Nabil

Verified by :

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Code: 0121-6767 Age: 38 y Date: 13/08/2024
Name: Nahlaa Shaaban Abd El Referred by Prof. Dr.: Abd El Ghany Mohamed Abd El
Hakeem Ghany

Bilateral Breast Sonomammography (Cranio-caudal & oblique)

The left breast :

Multiple Irregular hyper dense spiculated masses are seen in lower quadrant & retroareolar surrounded by architectural distortion

By US : Multiple Irregular hypoechoic masses are seen extending in lower outer quadrant at 5-7 o'clock about 5 cm from the nipple , the largest mass is taller than wider measures 2.6X2X1.8 cm in its maximum diameters at 5 o'clock with internal microcalcifications. a nearby satellite lesion 8x5 mm about 1 cm from the forementioned mass

Another sizable mass is seen at 7 o'clock measures about 13 x12 mm

They are highly vascular on Doppler study .

The Left axilla shows infiltrated lymph node with loss of hilum measures about 3.2x 2 cm no infra or supraclavicular nodes .

Right breast: shows normal fibro-glandular parenchyma (ACR-B). No definite mass is detected. No suspicious micro-calcification is seen.

- Normal sonographic appearance , Nonspecific axillary nodes

OPINION:-

- Left Lower Quadrant multifocal malignant masses with ipsilateral infiltrated nodes (BIRADS 5).

*Kindly Accept My Best Wishes,
Dr.Omnia Bakry MS c*